Mercy Health

Care first

Monday 15/3/21 10am

Mercy Mental Health
24 Hour Psychiatric Triage

Phone: 1300 657 259

9928 7444 Sylvia Water Clinic

mercyhealth.com.au

SUMMARY

Mercy UR: 2645287
Surname: McLean
First Name: Richard
DOB: 8/04/1973

4/03/2021 5:45:00 PM

9/03/2021

(address and contact details following discharge)
ibbin St Footscray VIC 3011
Phone: 0400 639 248

Treating Team
Ward: Clare Moore Building Inpatient Team
Address: Clare Moore Psych Werribee Mercy Hospital, 298 Princes Highway, Werribee, VIC 3030, WERRIB
Phone: 8754 3560
Fax: 8754 3570
Consultant Psychiatrist: Dr Michael Lograsso
Medical Officer: Dr Zixuan Wang
Primary Nurse:

General Practitioner
Name: Dr Gp Unknown
Clinic: Default Practice
Address:
Phone:
Fax:

Brook Intake clinician.

Principal Diagnosis
Adjustment Disorder

Additional Diagnoses
Schizophrenia
Cluster B personality traits

Complications

Suicide attempt via cutting of L) cubital fossa on 25/02/2021 requiring 8 day ICU admission

Operations/Procedures (if ECT include number of treatments):

L) cubital fossa sutured in ICU – sutures removed when returned to CMB

Reasons for Referral/Admission:
(From admission notes):

Mercy Health

DISCHARGE SUMMARY

Mercy UR: 2645287
Surname: McLean
First Name: Richard
DOB: 8/04/1973

47-year-old single Caucasian gentleman gentleman brought in by CATT team after a concerned friend report decline in Richard's mental state , living in a private rental. Richard reported that he had lost his job as an art therapist with NDIS recently.

Mode of referral:
Referred by concerned friend called the CATT team about his organized plan to commit suicide.
Richard's friend reported that she is worried that Richard's mental health may be declining as has noticed on his website that he is escalating in distress and listing suicide dates with increased "venom" towards former GP and health care professionals.

Presenting symptoms from interview/recent documentation:

- Richard has been admitted under an Assessment order. He repeatedly advises "You are admitting me against my will"
- Advises that his suicidal plan was to "bring attention and traction to the social and political injustice towards him" Advises his legal issues with his GPs and how all of them are conspiring against him along with his "malicious, toxic and narcissistic ex-boyfriend". He advises that his Ex-boyfriend is a "millionaire" with a lot of influence. Advises that his ex-boyfriend works for the ASIO (Australian security intelligence organization) and he believes they are spying on him. and that his ex-boyfriend is conspiring with everyone "he swore to destroy and ruin me"
- Richard advises that he was sexually abused by a neighbour and his family never supported him when he came out about it.
- He also described that his family was 'toxic' and that he had had a friend who had suicided in August 2018.
- Richard reported that he had voiced suicidal ideation to a particular GP 6 years ago and this doctor proceeded to prescribe him tablets which he could overdose on. He recorded this conversation and has since made a complaint against that doctor with AHPRA and has attempted to sue him for compensation.
- Richard has sought support from various agencies and ministers including Greg Hunt (Health minister) and the President of Black Dog institute (Gordon Parker) "They have all turned their back to me"
- Richard reported that a recent application for income protection had been denied.
- Advises that he is not suicidal at present
- He reported that a licences/allowance to be recognised as a therapist was declined by the APS.
- Richard expressed frustration over his experiences with 'the system' and that he has not been helped adequately.
- He did not report feeling depressed or a disturbance in sleep or appetite.
- Richard reported that he at times has 'other reality' experiences and that in the past he experienced visual hallucinations but he does not agree with the pathologising of these experiences as 'psychosis'. " I refused to tell you if I have hallucinations as you look at me through a DSM framework"
- It is reported in previous interviews and also the written submission that Richard provided that Richard's former partner's boss worked for ASIO which he feels may have negatively impacted on his plight. Richard does not currently feel persecuted by ASIO or any specific individual.
- He reported his goal as wanting to be 'valued'.
- He reported spending time with his dog and friends.

Stressors include:

- Litigation against his previous GP for medical malpractice
- 3 year legal battle with sexual abuse case (as a victim) about 3 years ago, went through many lawyers with minimal results
- Has had a best friend and another friend suicide

Past Psychiatric History:

- Documented history of schizophrenia, ADHD, depression.
- CMI contacts with triage
- Nil admissions
- Engaged with PP Dr Horgan for past few years, no longer currently seeing PP
- Previously supported by GP
- Previously engaged in psychotherapy

Past Medical History:

- Prostate problem – 'no one found anything'
- Skin lesion on head
- MSM – sexually active, practises safe sex

Forensic History:

- Nil

Legal History:

- 3 year legal battle with sexual abuse case (as a victim) about 3 years ago
- Litigation against his previous GP for medical malpractice

Treatment/Progress:

- Admitted to CMB on 24/02/2021
- Attempted suicide attempt via cutting of L) cubital fossa on 25/02/2021 with parts from a vape pen
- Reports attempted suicide with intent to die was due to having his dexamphetamine ceased on admission, not feeling appropriately listened to by staff, and being on a TTO
- Admitted to ICU between 25/02/2021 and 4/03/2021 (see ICU discharge summary)
- Received 1 on 1 nursing special after transfer back to CMB until 7/03/2021
- Remained in RCA from 5/3/21 to 9/3/21 wherein Richard denied further suicidal ideation, plans, or thoughts of self-harm, with nil further attempts, was future-focused and exhibited nil psychotic features
- Dexamphetamine dose decreased from 40mg to 25mg mane after collaborative discussion
- Nil other changes made to regular medications

Significant Laboratory Findings
FULL BLOOD EXAMINATION

Coll.Date:	28/02/21	01/03/21	05/03/21
Coll.Time:	06:35	07:45	09:35

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DISCHARGE SUMMARY

Mercy UR: 2645287
Surname: McLean
First Name: Richard
DOB: 8/04/1973

Lab.No: 31331326 31331519 47478099

				Units	Ref.Range
Haemoglobin:	81 **	88 **	117 *	g/L	(130-180)
WCC:	6.5	5.1	5.6	x10 ^9 /L	(4.0-11.0)
Platelets:	113 *	149 *	349	x10 ^9 /L	(150-450)
RCC:	2.56	2.75 **	3.67 *	x10 ^12 /L	(4.50-6.50)
PCV:	0.24 *	0.26 *	0.35 *	L/L	(0.40-0.54)
MCV:	95	94	97 *	fL	(80-96)
MCH:	32	32	32	pg	(27-32)
MCHC:	335	341	331	g/L	(320-360)
Neutrophils:	3.4	2.3	3.4	x10 ^9 /L	(2.0-8.0)
Lymphocytes:	2.6	2.1	1.6	x10 ^9 /L	(1.0-4.0)
Monocytes:	0.5	0.5	0.4	x10 ^9 /L	(0.0-1.0)
Eosinophils:	0.1	0.1	0.1	x10 ^9 /L	(0.0-0.5)
Basophils:	0.0	0.1	0.1	x10 ^9 /L	(0.0-0.2)

SERUM/PLASMA BIOCHEMISTRY

Coll.Date: 01/03/21
Coll.Time: 07:45
Lab.No: 31331519

				Units	Ref.Range
Sodium:	142	--	--	mmol/L	(135-145)
Potassium:	3.4 *	--	--	mmol/L	(3.5-5.2)
Chloride:	106	--	--	mmol/L	(95-110)
Bicarbonate:	28	--	--	mmol/L	(22-32)
Urea:	3.5	--	--	mmol/L	(2.3-7.6)
Est. GFR:	> 90	--	--	mL/min	(> 60)
Creatinine:	82	--	--	umol/L	(60-110)
Albumin:	29 *	--	--	g/L	(36-49)
Calcium:	1.96 *	--	--	mmol/L	(2.15-2.65)
Cor.Calcium:	2.18	--	--	mmol/L	(2.15-2.65)
Phosphate:	1.01	--	--	mmol/L	(0.75-1.50)
Magnesium:	0.76	--	--	mmol/L	(0.60-1.10)

BLOOD CULTURE

1 aerobic and 1 anaerobic bottle received.
COLLECTED: 7:45 am on 01/03/2021

CULTURE:
No growth at 48 hours

Culture continues but no further report will be issued unless bottles become positive.

BLOOD CULTURE

1 aerobic and 1 anaerobic bottle received.

Mercy Health

DISCHARGE SUMMARY

Mercy UR: 2645287
Surname: McLean
First Name: Richard
DOB: 8/04/1973

COLLECTED: 12:30 pm on 28/02/2021

CULTURE:
No growth at 48 hours

Culture continues but no further report will be issued unless bottles become positive.

SERUM IRON STUDIES

Date:26/02/21
Time:05:30
Lab.No:30318448

		Units	Ref. Range
Ferritin:	979	ug/L	(30-320)
Iron:	5	umol/L	(10-30)
Transferrin:	1.6	g/L	(2.0-3.6)
Transferrin Sat:	12	%	(13-47)

VITAMIN B12 AND FOLATE

Date: 26/02/21
Time: 05:30
Lab.No: 30318448

			Units	Ref. Range
Vitamin B12	:	348	pmol/L	(150-700)
Serum Folate	:	27.8	nmol/L	(> 9.0)

SERUM/PLASMA BIOCHEMISTRY

Coll.Date: 25/02/21
Coll.Time: 14:50
Lab.No: 31331022

					Units	Ref.Range
CPK:	99	--	--	--	-- U/L	(0-240)

Coll.Date: 25/02/21
Coll.Time: 14:50

Name of Test : CT ABDOMEN AND PELVIS WITH CONTRAST, CT BRAIN W/OUT CONTRAST, CT ANGIOGRAPHY - CHEST/ABDOMEN/UPPER LIMBS
UR: 2645287

DISCHARGE SUMMARY

McLean, Richard
2 McCubbin Street, FOOTSCRAY VIC 3011
Birthdate: 08/04/1973 Age: Y47 Sex: M
Addendum

ADDENDUM REPORT
Left arm

- The left subclavian artery, brachial, radial and ulnar arteries show normal
- contrast opacification.
- No pseudo aneurysm demonstrated.
- There are subcutaneous pocket of gas in the region of the cubital fossa, in
- keeping with the given history of laceration. There is a superficial vein
- coursing adjacent to this. There is soft tissue stranding, however no
- well-circumscribed haematoma is shown.

Addendum Report Reported by: Dr Neha Singh

Addendum electronically signed at 1:06 am Fri, 26th Feb 2021

ORIGINAL REPORT
CT of head, chest and abdomen and pelvis

CLINICAL DATA

Hypertension? Head strike question a source of infection

TECHNIQUE

IV contrast portal venous phase CT abdomen and pelvis performed.

FINDINGS

Head

- No intracranial bleed or surface fluid collection. Grey-white matter differentiation is preserved. No intracranial mass lesion or raised intracranial pressure. No hydrocephalus. No skull fracture or destructive bony lesion. No sinusitis or mastoiditis.

Chest

- No hydropneumothorax or pericardial effusion. No mediastinal collection. No
- consolidation, lung nodule, mass lesion, or bronchiectasis. Thoracic aorta is
- unremarkable.
- No lymphadenopathy.
- No PE.
- No fracture or bony lesion.

Abdomen and pelvis

McLean, Richard
2 McCubbin Street, FOOTSCRAY VIC 3011
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- unremarkable.
- No lymphadenopathy.
- No PE.
- No fracture or bony lesion.

Abdomen and pelvis

DISCHARGE SUMMARY

- IDC is in situ and bladder is empty.
- Right femoral vein catheter in place.
- No diverticulosis or enterocolitis.
- No mechanical bowel obstruction ischaemia or perforation.
- No appendicitis.
- No free peritoneal gas of fluid.
- No retroperitoneal or abdominal wall haematoma.
- Liver bile ducts gallbladder spleen adrenal glands both kidneys renal collecting
- systems and pancreas are unremarkable.
- Intra-abdominal vessels enhancing normally.
- No lymphadenopathy.
- No active GI bleed.

CONCLUSION
- No acute intracranial pathology.
- No intrathoracic or intra-abdominal source of infection identified.

Discharge Medications

Current Medication	Strength	Dosage	Frequency
Esomeprazole AN Tablets	40 mg	40mg	mane
APO-Flucloxacillin Capsules	500 mg	500mg	q.i.d.
APO-Diazepam Tablets	5 mg	5mg	p.r.n.
APO-Paroxetine Tablets	20 mg	20mg	mane
APO-Amisulpride Tablets	200 mg	600mg	nocte
Aspen Dexamfetamine Tablets	5 mg	25mg	mane
Truvada 300/200 mg Tablets	300 mg/200 mg	1	mane

Amount of Medication Supplied on Discharge (days/weeks):
7 days

Discharge and Follow Up Recommendations
- Discharge as voluntary patient
- Repeat FBE on 12/03/2021 with GP follow-up and monitoring of Hb
- Case management (referred to Saltwater clinic)

Discharge summary completion details:
Name: Dr Zixuan Wang
Designation: Registrar
Date: 9/03/2021